Van Pelt & Company

Public Comment on Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

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Infrastructure Enablement for Clinical AI — The role of a capacity data utility

The utility model underpinning the shared capacity data utility is intentionally flexible, supporting multiple care settings with distinct operational challenges while relying on a single, vendor-agnostic operational infrastructure.

In behavioral health settings, limited inpatient capacity, workforce shortages, and fragmented placement processes frequently result in prolonged emergency department boarding and inappropriate placements. Real-time operational visibility into behavioral health bed availability and staffing constraints enables faster, more appropriate placement decisions, reduces strain on emergency services, and improves outcomes for patients in crisis. Illustrative concepts for this care setting are available upon request.

In rural care environments, distance, limited access to specialty care, and staffing shortages require precise, time-sensitive decision-making about when patients can be safely managed locally and when timely transfer to higher-acuity care is essential. A capacity data utility supports these decisions by providing real-time visibility into system capacity without imposing additional reporting burden on already resource-constrained facilities. Illustrative rural care concepts are available upon request.

At the statewide level, governors, state health departments, and emergency management agencies require continuous situational awareness to support preparedness, surge response, and alignment with federal programs such as the Hospital Preparedness Program (HPP) and NHSN Bed Connectivity. A capacity data utility provides a shared operational view that supports both day-to-day coordination and emergency response. Statewide capacity concepts are available upon request.

Regional and multi-state coordination is increasingly critical for disaster response, National Disaster Medical System (NDMS) operations, and military-civilian medical surge scenarios. A capacity data utility supports interoperability across jurisdictional boundaries while preserving local control and governance.

Application Across Care Settings

Accelerating the adoption and effective use of artificial intelligence (AI) in clinical care requires confronting a reality that has become increasingly clear across federal, state, and provider environments: the primary constraint on meaningful AI deployment is not model capability, but the absence of shared, trusted, real-time operational data infrastructure that reflects how care is actually delivered across hospitals, regions, and states.

Over the past several years, federal investments have supported a wide range of digital health tools, dashboards, and reporting pipelines. Yet these efforts remain fragmented, episodic, and often disconnected from frontline decision-making. As a result, AI tools are frequently deployed as point solutions—optimized for narrow workflows but blind to system-level constraints such as staffing shortages, bed availability, specialty service access, or regional surge conditions. In these environments, even well-designed AI tools risk producing recommendations that are clinically reasonable but operationally infeasible.

The shared capacity data utility model directly addresses this foundational gap. A capacity data utility is designed as a vendor-agnostic, public-private model that leverages existing systems and non-PHI operational data to provide a continuously available view of healthcare system capacity. Importantly, this form of shared operational capacity infrastructure is not a new reporting requirement, centralized data warehouse, or prescriptive technology mandate. Instead, it functions as enabling infrastructure—providing the shared operational context required for AI tools, clinicians, administrators, and public agencies to act effectively in both routine and crisis conditions.

Practical Experience Informing the Capacity Data Utility Model

This utility-based approach is grounded in real-world experience working with states and health systems to operationalize real-time capacity and coordination across diverse care environments. In these efforts, success did not hinge on introducing new technologies; it depended on designing governance and operational models that competing providers would trust and actively use.

Across multiple state-level initiatives, the work involved convening competing hospitals, aligning state health departments and emergency management agencies, and integrating technology vendors without allowing any single platform to dominate. These environments required careful attention to neutrality, transparency, and shared value. Providers participated not because they were mandated to do so, but because the system's outputs helped them solve immediate operational problems—reducing transfer delays, improving patient placement, and anticipating system stress before it manifested as clinical harm.

A consistent lesson from this work is that sustainability depends on shared governance and operational relevance. When data is governed collaboratively, limited to non-PHI operational signals, and refreshed frequently enough to inform real decisions, providers are willing to participate and continue participating over time. Conversely, when systems are built primarily for compliance or retrospective reporting, adoption quickly erodes.

This type of shared capacity infrastructure reflects these lessons. Its design prioritizes federated data exchange, provider-led governance, and high-frequency operational signals. This approach lowers privacy risk, reduces regulatory friction, and aligns incentives across hospitals, states, and federal partners.

A capacity data utility as Enabling Infrastructure for Responsible Clinical AI

Clinical AI cannot be responsibly scaled without system-level context. AI tools that influence clinical decisions—whether related to triage, transfers, staffing, or care coordination—must account for real-time constraints across the care continuum. A capacity data utility provides this context by standardizing and sharing operational capacity signals that already exist within hospital systems but are rarely visible beyond institutional boundaries.

By doing so, a capacity data utility enables system-aware AI, allowing tools to incorporate real availability of beds, staff, and services into recommendations. This is particularly critical in emergency care, rural health, behavioral health, and disaster response, where delays and misalignment have disproportionate consequences.

Equally important, a capacity data utility creates a foundation for real-world evaluation and post-deployment monitoring of AI tools. Rather than relying solely on pre-deployment validation or static performance metrics, a capacity data utility enables continuous assessment of how AI-assisted decisions affect system-level outcomes such as boarding times, transfer delays, and access disparities. This supports a learning health system approach without requiring disclosure of proprietary algorithms.

Equity, Access, and Preparedness Implications

Absent shared operational infrastructure, AI tools risk reinforcing existing inequities by optimizing within well-resourced systems while leaving rural and safety-net providers behind. A capacity data utility mitigates this risk by ensuring that AI-enabled decisions reflect actual system capacity across all participating facilities, not just those with the most advanced digital infrastructure.

This approach aligns directly with HHS priorities related to health equity, rural access, and national preparedness. By focusing on non-PHI operational data and shared governance, a capacity data utility also strengthens trust and reduces barriers to participation among providers serving underserved populations.

Policy Implications for OneHHS

To accelerate responsible clinical AI adoption at a national scale, HHS should recognize that enabling infrastructure is as critical as algorithm development. A capacity data utility is a practical model for designing, governing, and sustaining such infrastructure without increasing regulatory burden or fragmenting existing investments.

Alignment of a capacity data utility to OneHHS RFI Themes

Within health care organizations, the adoption and sustained use of AI is most strongly influenced by operational and clinical leadership responsible for patient flow, staffing, emergency response, and care coordination. A capacity data utility supports these decision-makers by shifting governance from individual AI tools to shared, system-level operational governance structures, enabling consistent evaluation and accountability across institutions and jurisdictions.

This submission is intended to directly address the core themes raised in the OneHHS Request for Information on the adoption and use of artificial intelligence in clinical care. The shared capacity data utility model addresses these themes not by proposing new AI tools, but by describing the enabling infrastructure required for AI to be deployed safely, equitably, and at scale across diverse care settings.

The RFI highlights persistent barriers to AI adoption beyond technical capability. This response identifies fragmented, episodic, and non-operational data infrastructure as a primary constraint and describes a capacity data utility as a utility-style model designed to overcome these barriers by reusing existing systems and focusing on shared, real-time operational context.

The RFI emphasizes the need for interoperable data environments that support AI. A capacity data utility is intentionally vendor-agnostic, federated, and focused on non-PHI operational signals, enabling interoperability without introducing new privacy risk or duplicative reporting structures. This approach aligns with OneHHS objectives to reduce fragmentation while preserving local control.

The RFI seeks input on how AI should be evaluated once deployed in clinical environments. A capacity data utility supports real-world, post-deployment evaluation by linking AI-assisted decisions to system-level operational outcomes such as transfer delays, boarding, and access constraints, without requiring disclosure of proprietary algorithms.

Equity is a central concern of the RFI. This response explains how AI deployed without system-level context can exacerbate disparities, particularly for rural, safety-net, and behavioral health providers. A capacity data utility mitigates this risk by ensuring that AI-enabled decisions reflect actual capacity across all participating facilities, not just those with the most advanced digital infrastructure.

The RFI asks whether proposed approaches are adaptable across clinical and operational contexts. A capacity data utility is designed to function consistently across behavioral health, rural care, statewide coordination, and regional or multi-state environments, using the same underlying infrastructure while supporting setting-specific use cases.

Finally, the RFI emphasizes the importance of coordination across HHS operating divisions. A capacity data utility is presented as shared enabling infrastructure that supports multiple missions—clinical care delivery, preparedness, public health, and emergency response—without creating new silos or prescriptive mandates, aligning with the intent of a OneHHS approach.

Conclusion

The promise of clinical AI will not be realized through isolated tools or episodic data feeds. It requires shared operational infrastructure that reflects how care is delivered across systems and geographies. A capacity data utility offers an experience-informed, scalable approach to meeting this need—enabling AI to improve care delivery, advance equity, and strengthen preparedness without increasing fragmentation or burden.

For clarity, references in this submission to capacity data utilities are intended to be illustrative of a general infrastructure approach, rather than an endorsement of any single organization, platform, or implementation model.

Appendix (Reference)

Additional background materials are available upon request, including a capacity data utility concept, illustrations for behavioral health, rural care, statewide capacity coordination, and regional or multi-state implementation models. These illustrative concepts are intended to demonstrate operational use cases and are not prescriptive requirements for technology.

Legislative Context and Policy Alignment

The infrastructure approach described in this submission aligns with several legislative vehicles under consideration or enacted during the 119th Congress that could support implementation without creating new regulatory mandates.

First, H.R. 2936 establishes a framework to improve real-time visibility into hospital capacity and preparedness coordination. A capacity data utility functions as an execution layer for this policy intent by providing shared, vendor-agnostic operational infrastructure that enables states and regions to operationalize capacity awareness using existing systems and non-PHI data.

Complementing this approach, S. 1974, currently under consideration in the Senate, advances similar objectives by reinforcing the federal role in strengthening the healthcare system's readiness, situational awareness, and coordination across care settings. The bill's emphasis on preparedness, continuity of operations, and cross-sector alignment underscores the need for shared operational infrastructure that can be used consistently by states, providers, and federal partners. A capacity data utility directly supports this intent by enabling standardized, real-time operational visibility without prescribing specific technologies or imposing new reporting requirements.

Finally, provisions included in the National Defense Authorization Act (NDAA) signed into law by President Trump reinforce the importance of medical readiness, surge capacity, and civilian-military coordination. A capacity data utility aligns with this direction by providing a reusable operational backbone that can support preparedness, response, and recovery missions across HHS, DoD, and state partners.

Taken together, H.R. 2936, S. 1974, and the NDAA provide complementary policy signals and potential funding pathways that could support the development and scaling of shared capacity infrastructure—such as a capacity data utility—while preserving flexibility for states, providers, and federal agencies to implement solutions appropriate to their operational context.